policies, community initiatives, and more robust data preserving the voice of gender diverse patients.

Abstract

Transgender and gender diverse individuals experience 51% higher mortality rates and elevated incidence of serious illnesses including HIV/AIDS, cardiovascular disease, and lung and hematological cancers compared to cisgender populations(1). Despite these disparities, discrimination and stigma continue to create barriers to high quality hospice and palliative care(2,3).

This scoping review identified the unmet needs of gender diverse adults receiving serious illness care in the United States. Following PRISMA-ScR guidelines (4), we systematically searched eight databases (PubMed, CINAHL, PsycINFO, Web of Science, Embase, Scopus, ProQuest Dissertations, and grey literature sources) in June 2025. Inclusion criteria required: English-language publications from U.S. settings, exclusive focus on gender diverse adults, and examination of unmet needs in serious illness, hospice, or palliative care contexts. Two reviewers independently screened titles, abstracts, and full texts.

Only 10 publications met eligibility criteria, reflecting significant gaps in this literature. Thematic analysis, informed by minority stress theory(5), identified five categories of unmet needs: (1) competent healthcare professionals with gender-affirming training; (2) holistic, inclusive care delivery models; (3) family and chosen family engagement protocols; (4) protective institutional policies and legal frameworks; and (5) rigorous, patient-centered research.

Critical limitations include the small sample size despite comprehensive searching; predominance of healthcare professional perspectives rather than transgender patient voices; reliance on descriptive rather than rigorous empirical studies; and absence of intersectional analyses examining race, class, and disability. These limitations fundamentally constrain generalizability and clinical application.

Implications: This review exposes substantial underresourcing of research on gender diverse patients' end-of-life needs. Urgent priorities include: systematic competency training for hospice and palliative care professionals, development of evidence-based clinical guidelines, advocacy for protective legal frameworks, and most critically, conducting robust qualitative and quantitative research centering transgender patients' lived experiences and priorities in serious illness care.

Keywords. Symptom management, Pediatrics, Quality improvement, Communication.

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Upskilling Palliative Care Across the Community

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Outcomes:

1. Replicate and critically adapt the four described projects to meet the specific palliative care needs of their communities.

2. Identify 3 pitfalls in community project implementation and apply preplanning strategies to avoid them, based on real-world examples.

Key Message: Palliative care community projects implemented across Hawai'i educated hundreds of frontline providers and expanded the palliative care network to include community health workers and emergency medical technicians. Increased access to medical information provided via videos and enhanced communication skills for providers will hopefully result in improved patient/provider shared decision making.

Abstract

Palliative care (PC) offers an appropriate, and affordable solution for persons with serious illness.1 Despite its benefits, communities are facing challenges due to access, workforce limitations and lack of training.

Nine community projects were designed to support and expand the ability of PC interdisciplinary teams to deliver home-based PC across Hawai'i.

Four projects implemented from March 1, 2024, to March 31, 2025, demonstrated synergistic improvements.

1: Nurse Apprentices: Four new graduated, locally recruited nurses, were hired by a rural hospice/PC organization and mentored to provide care for patients/families.

2: Enhanced Communication Skills: Staff from varied roles, participated in nationally validated PC communication courses. Pre/post-training surveys measured comfort and skills improvements.

3: Educational Videos for Patients/Families: Multilingual, evidenced-based PC videos were implemented to support patient/family goals of care discussions. Video views and satisfaction surveys were collected.

4: Foundational Education: Basic PC education was provided to community health workers (CHWs), and emergency medical technicians (EMTs) including faculty and students. Knowledge assessments were conducted before and after training.

Results: 1: Four nurses completed mentorship, are independently caring for patients, and plan to take national PC certification exam.

2: Trained reached 14 new faculty, 37 participated in an eight-hour session with role play, and over 200 participated in basic empathy skills. Skills and comfort improved significantly in 6 of 7 areas.

3: Frontline staff and administrators (N>540) across 18 organizations received training on integrating videos into clinical care. Over 1,228 videos viewed by staff and patients/families across 18 organizations. While only 25 surveys were returned, all respondents would recommend the videos.

4: Over 250 participants from 45 organizations attended 21 classes. Four-hour basic or 6-hour instructors courses included 87 EMTs and 134 CHWs. Thirty instructors completed the course, 11 can independently teach the course. All pre/post knowledge changes were statistically significant (95% CI: 2.76%-6.74%).

Keywords. Pain management, Symptom management, Onoclogy, Caregivers, Bereavement, DEIJ, Hospice, Communication.

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Using Implementation Science Methods to Develop and Test a Buprenorphine Pocket Card and Tool Kit

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Outcomes:

1. Upon completion of the session, participants will have a comprehensive understanding of the development and impact of the Buprenorphine Pocket card and Tool Kit on palliative care clinicians' prescribing confidence and knowledge.

2. Upon completion of the session, participants will apply the learned knowledge by disseminating the Buprenorphine Pocket card and Tool Kit at their institution to increase buprenorphine knowledge and prescribing confidence of palliative care interdisciplinary team members.

Key Message: Buprenorphine is an evidence-based opioid option for patients with serious illness, but it is underused. Using the principles of implementation science, we developed, distributed, and tested a Buprenorphine Pocket Card and Tool Kit. The Buprenorphine Pocket Card and Tool Kit increased knowledge and prescribing confidence and is ready for dissemination.

Abstract

Background. Buprenorphine is an underused opioid therapy for patients with serious illness (1,2). Using implementation science methods (3,4), we developed, distributed, and tested a Buprenorphine Pocket Card and Tool Kit (Bupe PC&TK).

Objectives. We assessed baseline buprenorphine learning needs, knowledge, confidence, and ascribed importance. Then we tested the impact of the Bupe PC&TK, as an implementation strategy, using a pre-/post-study design.

Methods. We conducted a national online mixed methods survey (March 2024- June 2024) at baseline and six weeks following distribution of a Bupe PC&TK. Palliative care clinicians were recruited through palliative care networks and snowball sampling.